Royal Devon University Healthcare NHS Foundation Trust

ENVIRONMENTAL VENTILATION POLICY	
Post holder responsible for Procedural Document	Head of Estates
Author of Policy	Head of Estates
Division/ Department responsible for Procedural Document	Estates and Facilities Management/Estates
Contact details	*X3110*
Date of original document	05/10/2018 (East) 01/06/2018 (North)
Impact Assessment performed	**Yes**
Ratifying body and date ratified	Infection Prevention & Decontamination Assurance Group *& 10 February 2025*
Review date	*August 2029*
Expiry date	*February 2030*
Date document becomes live	21 May 2025

Please *specify* standard/criterion numbers and tick ✓ other boxes as appropriate

Monitoring Information		**Strategic Directions – Key Milestones**	
Patient Experience		Maintain Operational Service Delivery	
Assurance Framework		Integrated Community Pathways	
Monitor/Finance/Performance		Develop Acute services	
CQC Fundamental Standards - Regulation**:**		Infection Control	
Other *(please specify)*:			
Note: This document has been assessed for any equality, diversity or human rights implications			

Controlled document
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Full History		**Status: Draft** or **Final**	
Version	Date	Author	Reason
1	05/10/2018	Head of Estates	New Policy
2	11/11/2021	Deputy Head of Estates	Review Minor amendments and changes to HTM guidance.
3	05/07/2024	Head of Estates	Review of both policies to become Trust wide.

Associated Trust Policies/ Procedural documents:	*Management of Estates Contractors SOP* *Risk Assessment Policy* *Health and Safety Policy* *Fire Safety Policy* *Infection Control in the Built Environment*
Key Words	*Ventilation* *Infection Control*
In consultation with and date: Estates Officers (East and North) – 28 August 2024 Estates Water Safety and Ventilation Group – 28 August 2024 Estates Governance Group – 15 October 2024 Governance Managers - 15 October 2024 Divisional Directors 15 October 2024 Associate Medical Directors - 15 October 2024 Assistant Directors of Nursing - 15 October 2024 Senior Nurses - 15 October 2024 Quality Assured – 25 November 2024 Trust Water Safety and Ventilation Group **– 20 January 2025** Infection Prevention and Decontamination Assurance Group (ratified) – 10 February 2025	
Contact for Review:	Head of Estates
Executive Lead Signature: *(Applicable only to Trust Strategies & Policies)*	**Hannah Foster, Chief People Officer**

CONTENTS

KEY POINTS OF THIS POLICY: 4
1 INTRODUCTION 5
2 PURPOSE 5
3 DEFINITIONS 7
4 DUTIES AND RESPONSIBILITIES OF STAFF 8
5 PROCESSES AND PROCEDURES 10
6 ARCHIVING ARRANGEMENTS 13
7 PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY 14
8 REFERENCES 14
APPENDIX 1: COMMUNICATION PLAN 16
APPENDIX 2: EQUALITY IMPACT ASSESSMENT TOOL 17

KEY POINTS OF THIS POLICY:

Ventilation is used extensively in all types of healthcare premises to provide a safe and comfortable environment for patients and staff. More specialised ventilation is provided in areas such as operating departments, critical care areas and isolation facilities for patient treatment.

Ventilation is also installed:

- To ensure compliance with the quality assurance requirements of items processed in pharmacies and sterile services departments.
- To protect staff from harmful organisms and toxic substances (for example in laboratories).

Implementation of this policy will:

- Ensure that ventilation equipment is suitable for its intended use and is maintained to satisfactory performance levels.
- Contribute to the overall control of infection agenda within the Trust.
- Comply with health and safety legislation requirements.
- Ensure the standards set out initially by Standards for Better Health and modified over time by the Care Quality Commission are met.
- Maintain the health, comfort and environment for all patients, staff and visitors to the Trust by ensuring adequate ventilation exists and it is fully functional.

1 INTRODUCTION

1.1 This document sets out the Policy for managing air handling systems and ventilation systems at the Royal Devon University Healthcare NHS Foundation Trust (hereafter referred to as 'the Trust').

1.2 The Trust management recognises its commitment to provide a safe, high quality healthcare environment for its patients, staff and visitors. This is partially achieved by ensuring that Trust premises are maintained to a high standard, comply with statutory and mandatory requirements and best practice as set out in Health Technical Memorandums and other Department of Health (DH) guidance.

1.3 This document describes the systems and processes required to ensure that air handling systems are maintained, inspected and tested to provide a safe and suitable environment. The policy is based on HTM 03-01 Part A and Part B (previously HTM 2025), local building regulations and relevant health and safety requirements.

1.4 Ventilation is used extensively in all types of healthcare premises to provide a safe and comfortable environment for patients and staff. More specialised ventilation is provided in areas such as operating departments, critical care areas and isolation facilities for patient treatment.

1.5 Ventilation is also installed:

- To ensure compliance with the quality assurance requirements of items processed in pharmacies and sterile services departments.
- To protect staff from harmful organisms and toxic substances (for example in laboratories).

1.6 The policy describes the roles and responsibilities of those involved in the design, management, operation, maintenance and testing of systems and the key associated activities. It applies to all properties owned and maintained by the Trust including properties leased, rented or occupied under lease or other occupancy agreement. It covers maintenance on all ventilation/air conditioning plant and associated installations (such as duct work and fire dampers) undertaken by the Estates Department.

1.7 The Trust will seek assurance from the landlord of properties that are leased with non-repairing leases, such as NHSPS, regarding their ventilation compliance controls. This assurance will be discussed and managed through the Water and Ventilation Safety Group (WVSG).

2 PURPOSE

2.1 The Trust has an objective of providing safe, high quality healthcare environments for its patients, staff and visitors. This is achieved by ensuring that Trust premises are designed and maintained to high standards and comply with all the statutory and mandatory requirements as well as NHS technical standards.

This policy recognises that ventilation plant installed in trust premises is of varying age (some in excess of 40 years) and may not achieve the current regulatory standards. In these situations, the trust will maintain the equipment to the minimum

standards set out in HTM 03-01 part B and performance to at least the standards applicable at the time the plant was installed. The trust will plan to bring all ventilation plant up to current standards where reasonably possible.

2.2 Ventilation is used extensively in healthcare premises for both the comfort of occupants in buildings and to closely control the environment and air movement within the space. It is also used as controls to reduce hazards to patients and staff form airborne contaminants, dust and harmful microorganisms.

2.3 The link between surgical site infection and theatre air quality has been well established. If the ventilation plant has been installed to dilute or contain harmful substances, its failure may expose people to unacceptable levels of contamination. In addition, general ventilation systems which do not achieve and maintain the required standards can result in increased health risks to patients and other occupants.

2.4 The Trust is committed to ensuring that all ventilation/air conditioning equipment is installed, inspected, serviced and maintained in accordance with all of the relevant legislation and guidance to ensure that such equipment does not pose a health or operational risk to either, staff, patients or members of the public.

2.5 This document sets out the Trusts framework for achieving compliance with the following legislation and guidance:

- Health and Safety at Work Act 1974
- Control of Substances Hazardous to Health (COSHH) Regulations 2002
  Health Technical Memorandum 03-01 'Specialised ventilation for healthcare premises' Part A
  Health Technical Memorandum 03-01 'Specialised ventilation for healthcare premises' Part B

- Health and Safety Commission's Approved Code of Practice and guidance document 'Legionnaires' disease: the control of Legionella bacteria in water systems' (L8).
- Health Technical Memorandum 04-01 – 'The control of Legionella, hygiene, "safe" hot water, cold water and drinking water systems
- Building Regulations Approved Document F - Ventilation'.BS EN 15780:2011 – Ventilation for buildings – ductwork – cleanliness of ventilation systems
- Building & Engineering Services Association Guide to Good Practice - Internal Cleanliness of Ventilation Systems TR19 second edition 2012
- HSE Guidance on Ventilation and Air Conditioning during Coronavirus
- CIBSE Covid-19 Ventilation Guidance (copy on Estates Shared Drive)

2.6 Application of the arrangements set out in this policy will ensure that the Trust meets its statutory obligations and operates within approved safety standards and codes of practice.

2.7 Implementation of this policy will:

- Ensure that ventilation equipment is suitable for its intended use and is maintained to satisfactory performance levels.
- Contribute to the overall control of infection agenda within the Trust.
- Comply with health and safety legislation requirements.

- Ensure the standards set out initially by Standards for Better Health and modified over time by the Care Quality Commission are met.
- Maintain the health, comfort and environment for all patients, staff and visitors to the Trust by ensuring adequate heating and ventilation exists and it is fully functional.

3 DEFINITIONS

3.1 **Ventilation** is a means of removing and replacing the air in a space. In its simplest form this may be achieved by opening windows and doors. Mechanical ventilation systems provide a more controllable method. Basic systems consist of a fan and collection of distribution ductwork; more complex systems may include the ability to heat and filter the air passing through them.

3.2 **Air-conditioning** is the ability to heat, cool, humidify, dehumidify and filter air. This means that the climate within a space being supplied by an air-conditioning plant can be maintained at a specific level regardless of changes in the outside air conditions or the activities within the space.

3.3 **Critical systems –** A ventilation system which when out of service would seriously degrade the ability of the premises to deliver optimal healthcare would be considered critical. This includes:

- Operating suites of any type including rooms used for interventional procedures and their recovery areas.
- Airborne isolation facilities source and protection.
- Critical care units, intensive treatment or high-dependency unit.
- Neonatal unit and special care units.
- Invasive treatment, endoscopy and bronchoscopy rooms.
- Containment level 3 laboratory.
- Pharmacy aseptic suite.
- Inspection, assembly and packing room (IAP) in a sterile services department.
- MRI, CT and other types of imaging technologies that require stable environmental conditions to remain within calibration.
- Mortuary
- Any system classified as a Local Exhaust Ventilation (LEV) system under the COSHH regulations.
- Any other system that clearly meets the definition that “a loss of service from such a system would seriously degrade the ability of the premises to deliver optimal healthcare.

3.4 **Non-Critical systems –** General ventilation and extract systems in buildings other than those listed in para 3.3

3.5 **Authorising Engineer (Ventilation) -** A person appointed to provide independent auditing and advice on ventilation systems, to review documentation on verification and validation and witness the process as necessary.

3.6 **Authorised Person (Ventilation) -** A person appointed who manages the operation, maintenance and performance testing of all critical and non-critical ventilation systems.

3.7 **Competent Person (Ventilation) -** A person who undertakes work on systems as designated by Authorised Persons.

3.8 **Portable Air-conditioners** – a mobile air conditioning unit for emergency use in areas due to plant failure or extreme weather conditions.

3.9 **Portable recirculating filter units** – a mobile AHU to provide additional air filtration to reduce the risk of droplet and airborne infection.

4 DUTIES AND RESPONSIBILITIES OF STAFF

4.1 **The Chief Executive Officer** has overall responsibility for all activity, and must ensure that inspection, service and maintenance activities are carried out safely without hazard to staff, patients or members of the public.

4.2 **Chief Finance Officer** has responsibility to ensure that sufficient and suitable resources are in place to manage and maintain the Trust's ventilation equipment, including that suitably qualified personnel are employed to implement, manage and review this activity.

4.3 **The Joint Directors of Infection Prevention and Control** have responsibility for monitoring this policy as part of clinical assurance provision to the Trust Board and ensuring adequate management arrangements are in place.

4.4 **The Head of Estates (Designated Person)** has responsibility for regularly reviewing the Trust ventilation infrastructure and identify systems for replacement/upgrade through the Backlog Capital Programme (Estates Investment Plan). Implementing and reviewing this policy and ensuring that sufficient and competent staff and resources are available for all areas covered by this policy.

4.5 **The Head of Capital Projects** has the responsibility to ensure:

- All new installations meet the latest statutory requirements and technical standards including the provision of satisfactory validation reports;
- A suitably qualified person is involved in the design of all new installations and that commissioning and performance checks are undertaken and documented;
- That all new installations are accessible and maintainable without resorting to specialist access equipment or the need for removal of finishes/infrastructure;
- That maintenance teams are provided with comprehensive Operation and Maintenance manuals (O & Ms) on the handover of completed installations;
- Ensure that appropriate training and familiarisation is provided to in house, contract maintenance teams and system users.

4.6 **Authorising Engineer (AE)** will be appointed to undertake independent auditing and provide advice on ventilation systems. They must review and witness verification and validation documentation.

4.7 **Authorised Persons (APs) (ventilation)** will ensure that all ventilation/air conditioning equipment is inspected, serviced, maintained and tested in a safe manner without hazard to staff, patients or members of the public, including:

- Critical systems are identified and tested by a Competent Person;
- Ensure reactive and Planned Preventative Maintenance (PPM) arrangements are in place;
- Escalate any issues outside own area of authority;
- Maintain records of maintenance, testing and validation and verification work;
- Prepare and submit an annual performance report for the Water Safety and Ventilation Group.
- Ensure that all competent persons are suitably trained and aware of any hazards associated to the systems they are working on.

4.8 **Competent Persons (CPs)** are responsible for undertaking work on systems as designated by APs including:

- Carry out repairs and planned preventative routines as instructed by an AP and to provide feedback on performance and maintenance issues;
- To ensure all health and safety, COSHH, Trust policies and procedures and risk assessments are adhered to at all times.

4.9 **Local Managers** have a responsibility to report ventilation and system defects to the Estates helpdesk;

- Ensure that they are familiar with the function of and operation of the systems in their areas;
- Ensure reasonable access for maintenance purposes;
- Reporting all incidents (including near misses) via Datix web.
- Ensure that all authorised persons (ventilation) are made aware of any hazardous processes or substances associated with the systems

4.10 **The Estates Speciality Governance Group** has responsibility for providing advice and guidance on policy requirements; including proposing changes to this policy as and when new legislation or regulations are introduced. It will maintain an audit cycle for monitoring and review of compliance of this and other Estates policies within the Trust.

4.11 **Estates Water and Ventilation Safety Group (EWVSG)** will monitor and manage the safety systems in place to minimise risk from microbial and chemical hazards arising from the delivery and use of water and ventilation on Trust premises. This includes risk of infection from Legionella, Pseudomonas and other microorganisms. The group will provide assurance to WSVG that the Trust complies with relevant legislation and national guidelines.

4.12 **The Infection Prevention and Control (IPC) Team** is responsible for advice on monitoring infection control and microbiological performance of systems and provide infection control support to Estates staff.

4.13 **The Water and Ventilation Safety Group (WVSG)** has the following roles:

- Advising on the potential risk and identifying where systems do not comply with current guidance.
- Advising on the necessary continuing procedures for infection risk prevention.
- Monitoring the implementation and efficiency of the procedures.

- Approving and identifying any changes to those procedures.
- Monitoring and ensuring the maintenance of adequate verification records.

The Water and Ventilation Safety Group reports to the **Infection Prevention and Decontamination Assurance Group** which reports to the **Patient Safety Committee.**

4.14 Leased Properties - The Trust will seek assurance from the landlord of properties that are leased with no repairing leases, such as NHS Property Services, regarding their ventilation management controls. This assurance will be discussed and managed through the WVSG.

5 PROCESSES AND PROCEDURES

5.1 Standards

5.1.1 Technical and performance guidance is set out in Health Technical Memorandum 03-01 'Specialised ventilation for healthcare premises'. Where this is not applicable Approved Document F of the Building Regulations will be used.

5.1.2 In many existing systems, original design and commissioning information may not be available. It will therefore be necessary to determine a suitable level of system performance based on the function, purpose and age of the installation. This determined performance will be used as the standard for compliance as opposed to measured performance.

5.2 Design and installation

5.2.1 The design and installation of new equipment is fundamental to the long term provision of suitable systems which will perform well over a 15-20 year life span.

5.2.2 All ventilation/air conditioning equipment shall be appropriate for the area it is being designed for and it is only installed where absolutely necessary.

5.2.3 All ventilation/air conditioning equipment is to be designed and installed by suitably qualified personnel and complies with the requirements of HTM 03-01 and other mandatory and statutory legislation.

5.2.4 Systems shall be designed to the highest standards in terms of energy usage to ensure that the Trusts carbon and sustainability agenda are not compromised.

5.2.5 Cooling and/or air conditioning will only be installed in Trust buildings where there is a specific risk imperative or where critical plant needs to be maintained within prescribed temperature parameters or as stated within HTM requirements.

5.2.6 All installations should be easily accessible for maintenance and testing of all maintainable parts and components without the need for specialist access equipment and/or removal of building fabric and finishes.

5.2.7 When new equipment is accepted for use, full information as to its designed mode of operation together with maintenance procedures is provided as part of the handover procedures and prior to system use.

5.2.8 All new ventilation/air conditioning systems shall be commissioned and signed off by a suitably qualified person to ensure that the minimum standards are achieved.

5.2.9 The users and those who maintain the system need to be instructed in its safe operation and how to provide the correct environmental conditions as part of the handover procedure at the end of the commissioning period.

5.2.10 Portable air conditioners and Portable recirculating filter units will only be deployed under instruction from the IPC or site management teams, who must advise on any additional decontamination requirements of the units pre and post use, that are over and above the normal cleaning and maintenance routines.

5.3 Maintenance

5.3.1 All ventilation air handling units (AHU), plant, ductwork and systems shall be included in the planned preventative maintenance (PPM) system

5.3.2 Inspections and maintenance shall be carried out in accordance with the following:

- Heating and ventilation systems Health Technical Memorandum 03-01: Specialised ventilation for healthcare premises Part A & Part B.
- Health and Safety Commission's Approved Code of Practice and guidance document 'Legionnaires' disease: the control of Legionella bacteria in water systems' (L8).
- Health Technical Memorandum 04-01 – 'The control of Legionella, hygiene, "safe" hot water, cold water and drinking water systems'.
- BS EN 15780:2011 – Ventilation for buildings – ductwork – cleanliness of ventilation systems.
- Building & Engineering Services Association Guide to Good Practice TR19 - Internal Cleanliness and Hygiene Management of Ventilation Systems 2024 and Grease Specification for the Fire Risk Management of Grease Accumulation within Kitchen Extraction Systems 2020.
- The general frequency of inspections and verification for ventilation systems shall consist of:
    - All ventilation systems to be subject to inspection and maintenance on a 3 monthly cycle.
    - Ventilation systems serving critical care areas shall be inspected and maintained on a 3 monthly cycle with actual performance measured and verified annually.
    - Systems classified as an (LEV) system under HSG258 are to be examined and tested by a Competent Person every 14 months.
    - Kitchen Ventilation systems shall be inspected every 6 months and cleaned as necessary.
    - Pharmacy (Aseptic) Ventilation to be inspected every month and Service maintenance carried out on a 3 monthly cycle.

5.3.3 Air handling units should be vacuumed-out and/or washed down internally as necessary to remove obvious dust and dirt. The plant must not contain any material or substance that could support the growth of microorganisms.

5.3.4 Chiller batteries, humidifier units, energy-recovery batteries or plates and their drainage systems should be inspected annually and cleaned as recommended to remove visible contamination. Precautions should be taken to ensure no pooling of water has taken place prior to returning the unit to service as this will lead to proliferation of micro-organism leading to potentially harmful water/air-borne diseases.

5.3.5 Supply air distribution ductwork conveys air that has been filtered. It will require internal cleaning only when it becomes contaminated with visible dirt. The frequency of cleaning will depend on the age of the system and grade of the AHU final filter but will typically be in excess of ten years. There is no requirement to clean ductwork annually.

5.3.6 Extract air systems handle unfiltered air. They should be cleaned as frequently as necessary in order to maintain their operating efficiency. Room extract terminals, particularly those sited at low level in critical care areas, will need regular cleaning.

5.3.7 The methods stated in BS EN 15780 shall be used to determine whether systems require cleaning.

5.3.8 Split and cassette air-conditioning units incorporate internal recirculation air filters and a drainage system to remove condensate from the cooling coil. The systems should we inspected and cleaned every three months.

5.4 Testing

5.4.1 Commissioning and validation of new critical systems must be undertaken by the project in line with the process outlined in HTM 03-01 Part A including bacteriological sampling.

5.4.2. All critical ventilation systems should be inspected on a 3 monthly cycle and verified at least annually. In some circumstances the verification may need to be carried out more frequently subject to a risk assessment.

5.4.3 The 3 monthly cycle inspection should be a simple visual check; the annual verification will be a more detailed inspection of the system together with the measurement of its actual performance.

5.4.4 The annual verification is intended to establish that:

- the system is still required,
- the AHU conforms to at least the minimum standards and
- the fire containment has not been breached.

5.4.5 The verification should also assess whether the general condition of the ventilation system is adequate, the fabric of the area served is satisfactory and the system performance is adequate with respect to the functional requirement.

5.4.6 Annual verification will require:

• a full measure of the supply and extract air-flow rates;
• the calculation of room air-change rates if applicable;
• the measurement of room differential pressures if applicable;
• the measurement of room noise levels;
• air-quality checks if appropriate;
• a check on the control functions.

5.5 Information

5.5.1 The following information shall be provided adjacent to the plant to which it refers:

- General information regarding the intended operation of the plant together with a schematic diagram of the equipment and its distribution system.

- Specific information as to the purpose of the plant and details of those departments and/or personnel that should be informed prior to switching off or carrying out maintenance activities.

- Specific information required for the safety of the personnel carrying out the service and maintenance activities.

5.5.2 Ventilation system records and logbooks should be kept of the commissioning information, operational management routine, monitoring and maintenance;

5.5.3 A sign should be fitted indicating that 'No plant can be isolated without permission of the user' on AHU(s) identified as critical.

5.6 Records

5.6.1 In order that ventilation systems can be correctly operated and maintained it is essential that as-fitted drawings, operating manuals, maintenance instructions and commissioning manuals are available. Log books should be kept for each ventilation system consisting of maintenance records, test and validation data. These Log books can be in a digital format held on the estates shared drives.

5.7 Training

5.7.1 Competent Person carrying out maintenance of Ventilation Systems must receive suitable training, which includes information about any significant hazards arising due to their maintenance activities which may either affect them personally or any other person who may be affected by their actions or omissions. Training records shall be kept up to date for all staff.

6 ARCHIVING ARRANGEMENTS

The original of this policy, will remain with the author the Head of Estates, Estates Department. An electronic copy will be maintained on the Trust Intranet, P – Policies – *V – Ventilation Policy*. Archived electronic copies will be stored on the Trust's "archived policies" shared drive, and will be held indefinitely. A paper copy (where one exists) will be retained for 10 years.

7 PROCESS FOR MONITORING COMPLIANCE WITH AND EFFECTIVENESS OF THE POLICY

7.1 To evidence compliance with this policy, the following elements will be monitored:

What areas need to be monitored?	How will this be evidenced?	Where will this be reported and by whom?
All critical ventilation systems should be inspected quarterly and verified at least annually.	Quarterly inspections by local checklists Annual verification reports	Estates Monthly & Trust 6 Monthly Water Safety and Ventilation Group
Assurance of ventilation safety from properties not owned or managed by the Trust	Report to Water Safety & Ventilation Group	Estates Monthly & Trust 6 Monthly Water Safety and Ventilation Group

8 REFERENCES

- *Health and Safety at Work Act 1974*
  Available at: https://www.legislation.gov.uk/ukpga/1974/37

- *Control of Substances Hazardous to Health (COSHH) Regulations 2002*
  Available at: http://www.legislation.gov.uk/uksi/2002/2677/regulation/7/made

- *Health Technical Memorandum 03-01 'Specialised ventilation for healthcare premises'.*
  Available at:
  https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/144029/HTM_03-01_Part_A.pdf

- Health and Safety Commission's Approved Code of Practice and guidance document 'Legionnaires' disease: the control of Legionella bacteria in water systems' (L8).
  Available at: https://legionellacontrol.com/downloads/ACOP-L8-4th-Edition-Compliments-Legionella-Control-International.pdf

- Health Technical Memorandum 04-01 – 'The control of Legionella, hygiene, "safe" hot water, cold water and drinking water systems
  Available at:
  https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/524880/DH_HTM_0401_PART_A_acc.pdf

- Building Regulations Approved Document F - Ventilation'.BS EN 15780:2011 – Ventilation for buildings – ductwork – cleanliness of ventilation systems
  Available at:
  https://assets.publishing.service.gov.uk/government/uploads/system/uploads/attachment_data/file/468871/ADF_LOCKED.pdf

- Building & Engineering Services Association Guide to Good Practice TR19 - Internal Cleanliness and Hygiene Management of Ventilation Systems 2024 and Grease Specification for the Fire Risk Management of Grease Accumulation within Kitchen Extraction Systems 2020.

- HSE Guidance on Ventilation and Air Conditioning during Coronavirus
- CIBSE Covid-19 Ventilation Guidance (copy on Estates Shared Drive)

APPENDIX 1: COMMUNICATION PLAN

The following action plan will be enacted once the document has gone live.

Staff groups that need to have knowledge of the strategy/policy	Estates Staff Management Leads Department Heads
The key changes if a revised policy/strategy	Minor changes, policy review and amalgamated with Northern Devon policy.
The key objectives	The Trust has an objective of providing safe, high quality healthcare environments for its patients, staff and visitors. This is achieved by ensuring that Trust premises are designed and maintained to high standards and comply with all the statutory and mandatory requirements as well as NHS technical standards. It is to be noted that some of Air Handling Plant on our sites were installed to regulations previous to that which is current. Some plant is 20 years old and some is in excess of 40 years. It is recognised that in this case verification may only conform to that which the original equipment was installed to. Ventilation is used extensively in healthcare premises for both the comfort of occupants in buildings and to closely control the environment and air movement within the space. It is also used as, controls to reduce hazards to patients and staff form airborne contaminants, dust and harmful microorganisms.
How new staff will be made aware of the policy and manager action	*Cascade by email from manager, induction process*
Specific Issues to be raised with staff	*None*
Training available to staff	*Training is available for Estates Staff*
Any other requirements	*None.*
Issues following Equality Impact Assessment (if any)	*None*
Location of hard / electronic copy of the document etc.	The original of this policy, will remain with the author the Head of Estates, Estates Department. An electronic copy will be maintained on the Trust Intranet. Archived electronic copies will be stored on the Trust's “archived policies” shared drive, and will be held indefinitely.

APPENDIX 2: EQUALITY IMPACT ASSESSMENT TOOL

Name of document	Ventilation Policy
Division/Directorate and service area	Estates and Facilities Management/Estates Dept
Name, job title and contact details of person completing the assessment	Steve Cross, Head of Estates, Tel 3110
Date completed:	28 August 2024

The purpose of this tool is to:

- **identify** the equality issues related to a policy, procedure or strategy
- **summarise the work done** during the development of the document to reduce negative impacts or to maximise benefit
- **highlight unresolved issues** with the policy/procedure/strategy which cannot be removed but which will be monitored, and set out how this will be done.

1. What is the main purpose of this document?

The Trust has an objective of providing safe, high quality healthcare environments for its patients, staff and visitors. This is achieved by ensuring that Trust premises are designed and maintained to high standards and comply with all the statutory and mandatory requirements as well as NHS technical standards.
It is to be noted that some of Air Handling Plant on our sites were installed to regulations previous to that which is current. Some plant is 20 years old and some is in excess of 40 years. It is recognised that in this case verification may only conform to that which the original equipment was installed to.

Ventilation is used extensively in healthcare premises for both the comfort of occupants in buildings and to closely control the environment and air movement within the space. It is also used as, controls to reduce hazards to patients and staff form airborne contaminants, dust and harmful microorganisms.

The link between surgical site infection and theatre air quality has been well established. If the ventilation plant has been installed to dilute or contain harmful substances, its failure may expose people to unacceptable levels of contamination. In addition, general ventilation systems which do not achieve and maintain the required standards can result in increased health risks to patients and other occupants.

The Trust is committed to ensuring that all ventilation/air conditioning equipment is installed, inspected, serviced and maintained in accordance with all of the relevant legislation and guidance to ensure that such equipment does not pose a health or operational risk to either, staff, patients or members of the public.

2. Who does it mainly affect? *(Please insert an "x" as appropriate:)*

Carers ☒ Staff ☒ Patients ☒ Other (please specify) Visitors

3. Who might the policy have a 'differential' effect on, considering the "protected characteristics" below? (By *differential* we mean, for example that a policy may have a noticeably more positive or negative impact on a particular group e.g. it may be more beneficial for women than for men)

Please insert an "x" in the appropriate box *(x)*

Protected characteristic	Relevant	Not relevant
Age	☐	☒
Disability	☐	☒
Sex - *including: Transgender, and Pregnancy / Maternity*	☐	☒
Race	☐	☒
Religion / belief	☐	☒
Sexual orientation – *including: Marriage / Civil Partnership*	☐	☒

4. Apart from those with protected characteristics, which other groups in society might this document be particularly relevant to… (e.g. those affected by homelessness, bariatric patients, end of life patients, those with carers etc.)?

None

5. Do you think the document meets our human rights obligations? ☒

Feel free to expand on any human rights considerations in question 6 below.

A quick guide to human rights:
• ***Fairness –*** *how have you made sure it treat everyone justly?* • ***Respect –*** *how have you made sure it respects everyone as a person?* • ***Equality –*** *how does it give everyone an equal chance to get whatever it is offering?* • ***Dignity –*** *have you made sure it treats everyone with dignity?* • ***Autonomy –*** *Does it enable people to make decisions for themselves?*

6. Looking back at questions 3, 4 and 5, can you summarise what has been done during the production of this document and your consultation process to support our equality / human rights / inclusion commitments?

Any part of the document that favoured a particular group of people has been removed and judgements will now be made based on risk.

7. **If you have noted any 'missed opportunities', or perhaps noted that there remains some concern about a potentially negative impact** please note this below and how this will be monitored/addressed.

"Protected characteristic":	
Issue:	
How is this going to be monitored/ addressed in the future:	
Group that will be responsible for ensuring this carried out:	